ENSIGN SERVICES

POSTette: The Rehab Screen /Consultation Series: Overview

Reference: Rehabilitation P&P Policy 200; Medicare Benefit Manual Chapters 8 & 15

Understanding the Screen and Consultation Process:

Clinicians will perform routine and requested screens/consultations received verbally, in writing or based on an assessment schedule (MDS, Care Plan, etc.) The clinician will document the results of the screen/consultation and provide feedback to referral sources.

Requested therapy consultations outside of the normal screen schedule should have documentation in the clinical record to support the need for a therapy consultation (i.e. change in condition).

Purpose:

The purpose of a consultation/screen is to determine the need for a skilled evaluation and to make an impact on a resident's quality of life. Requests for consultation may be made by the physician, non-physician practitioner (NPP), nursing staff, other healthcare professionals, or family members.

Only clinicians will conduct the screen unless specific state practice acts are reviewed and approved for a different practice. Routine screens can be established by following MDS schedules, follow up from therapy discharge, establishing the appropriateness of Restorative Programming, etc.

Process:

Refer to Rehabilitation Policy and Procedure 200 (Rehab Screen / Consultation) for details.

Remember: If there is a request for an unscheduled routine screen it means something has changed in a resident's condition. The longer therapy waits to consult the longer a resident's quality of life could be impacted.

Routine screens need to be completed timely based on a schedule.

All Consultations/Screens will be logged/ documented with feedback provided to all referral sources.

The Do's and Don'ts

DO:

A consultation generally includes the following:

- A brief chart review
- A verbal consultation with the referring party, staff, resident and/or responsible party. Conduct a brief interview to determine prior functional level and any subsequent changes
- Observing another caregiver interact with the resident (**NOTE**: Observation should only occur if it is safe and appropriate)

DON'T

A consultation (or screening) does not include any method of "hands on" assessment of the patient or making any recommendations of care based on what is observed.

If more information is needed to determine what is going on….. Request an Evaluation.

Recommendations for equipment, change in exercise, change in ADL routine, change in diet, cannot be made within a consult/screen since this would require a skilled assessment.

Making recommendations based on a screen could be interpreted as practicing without a physician's order. If enough information was obtained during a screen to make recommendations, then that could be considered a non-covered evaluation/assessment without a physician's order.

In addition, Patient / Caregiver education cannot be completed within a consult/screen since it is defined as skilled intervention.

Documentation:

Each discipline needs to use the same process for capturing screen information. Screen information needs to be added to the Clinical Record and should include:

- A brief history relative to the current concern and/or problem
- Any identified changes in Prior level of function as it relates to the reason for consult
- Who requested the consult and why
- The determination of whether an evaluation is needed

If an evaluation is indicated, follow policy and procedures for obtaining an evaluation and treat order.

If an evaluation is not indicated, the documentation of the screen should indicate the reasons why an evaluation is not indicated at this time.

Remember: A Screen or Consultation is an opportunity to determine if and evaluation and/or further therapy services can improve a resident's quality of life.

Resources:

- Rehab Screen / Consultation; Patient Identification; Quality Indicator Reports; & Understanding Federal (F) Tags POSTettes

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